When Caring Backfires:
Experimental Evidence from Family Caregiving in India

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NEUDC 2026

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  - **Today:** Focus on demand-side barriers to take-up of preventative healthcare

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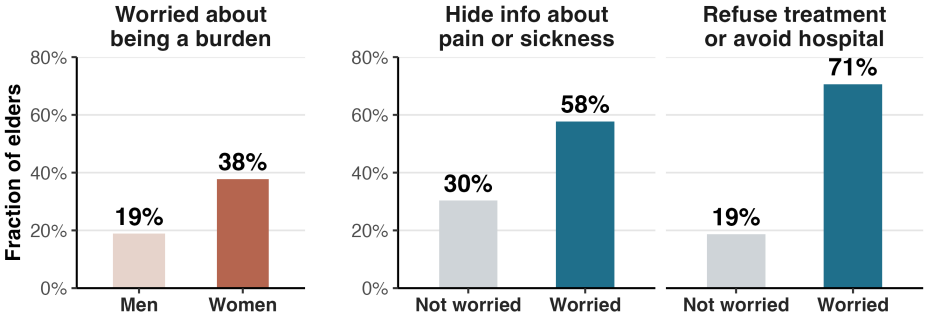
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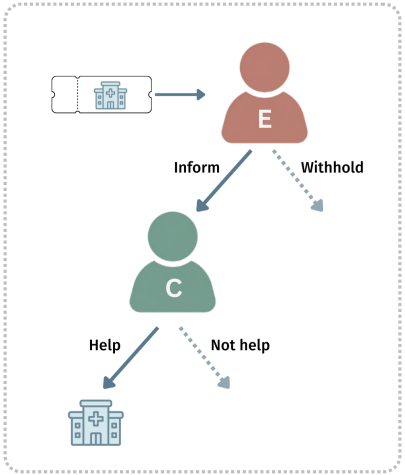


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  - Family caregivers often provide daily support
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- **Our paper:** Does worry lead to information frictions in the family?
  - This can decrease take-up of care, even when families value it
  - Key intuition: When there is pressure, sharing information is not free

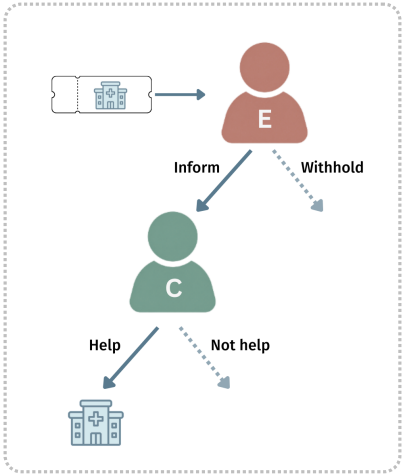
Our paper in two pictures

Only elder knows (Status quo)



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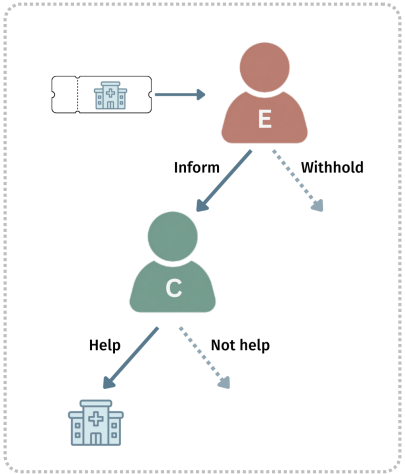
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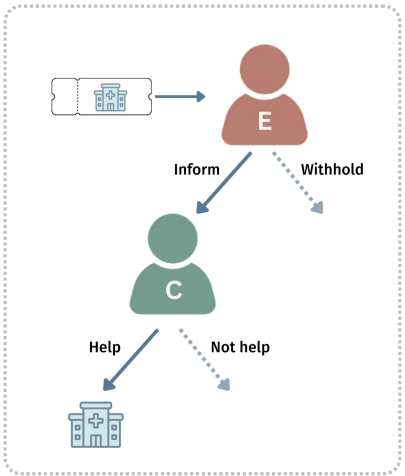
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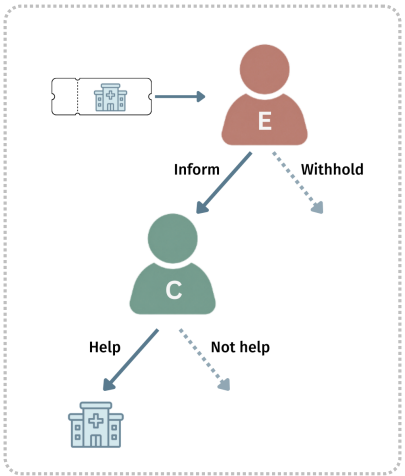
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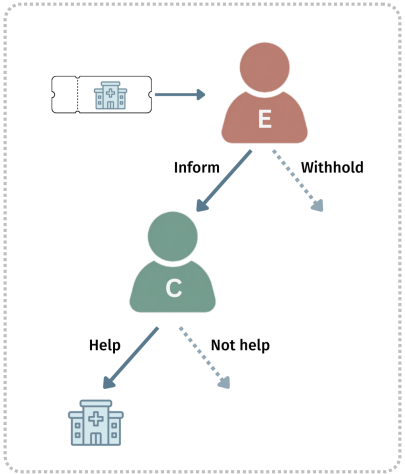
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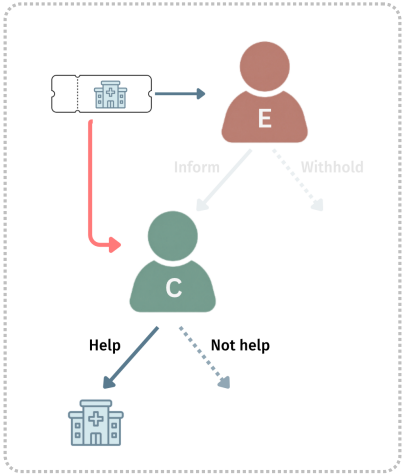
- Elder needs help to attend visit
- But informing may induce pressure
- So she may worry and not inform
  - Worry = care + pressure
- Even if caregiver would help!

Our paper in two pictures

Only elder knows (Status quo)



Both know (Treatment)



Main findings

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Worried elders forgo care that both elders and caregivers value

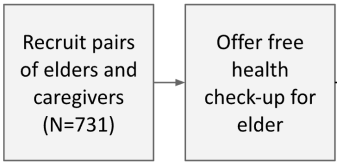
Setting: Elderly women and their caregivers in Tamil Nadu, India (N=731 pairs)

- Elders in our study are women aged 60-75 in rural Tamil Nadu, India
  - **Worried**: 20% report ever forgoing treatment out of worry about burden
- Caregivers are family members who live nearby and support in health tasks
  - Primarily sons (43%) and husbands (21%)



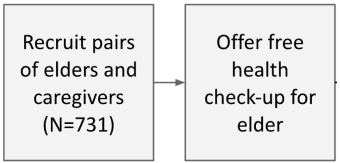


We partner with a private hospital to offer elders a free cardiac check-up



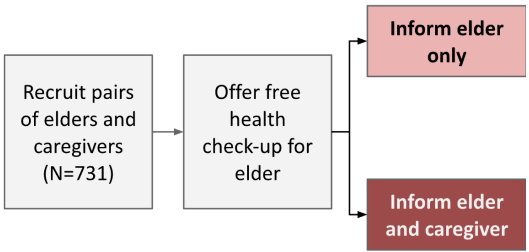
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- Invites conducted separately, privately
  - Paper voucher with details for elders
  - Sign-up: Caregiver must call by deadline

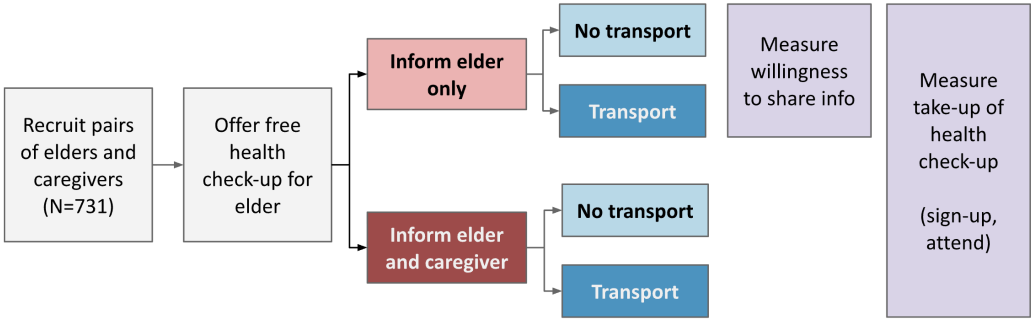
Randomize who in household is informed about check-up



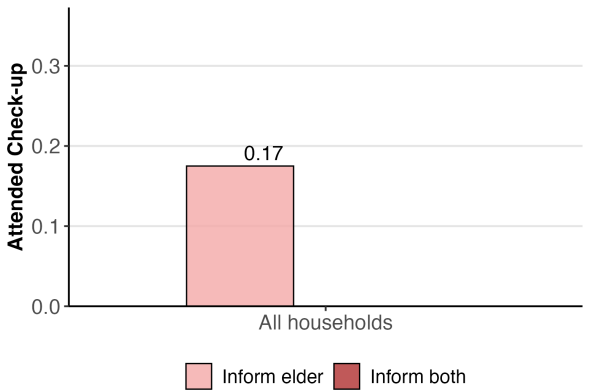
Measure take-up of health check-up

(sign-up, attend)

Cross-randomize how costly the check-up is (for caregivers)



Without information frictions, there should be no effect of informing caregiver

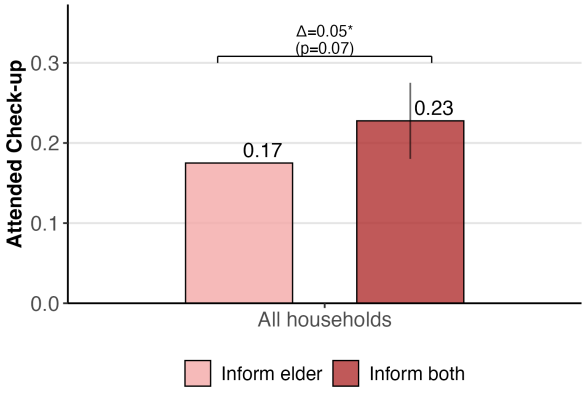


Effects are **large** in magnitude:

26% increase in sign-up rates and 30% in attendance

Effect is 70-73% of effect for free transport intervention

Informing caregiver about check-up increases take-up by 5pp

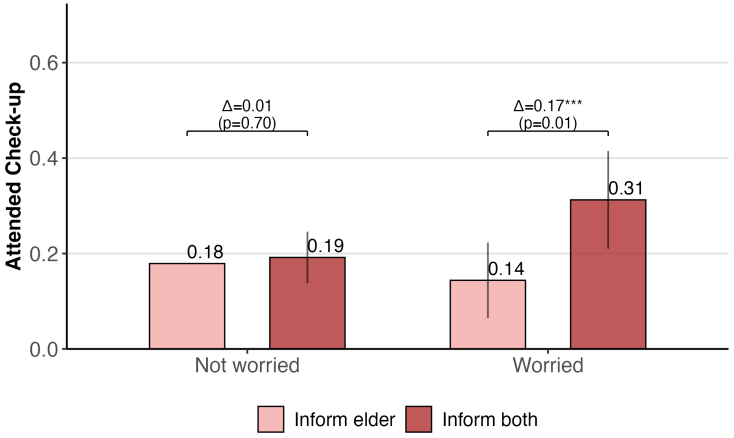


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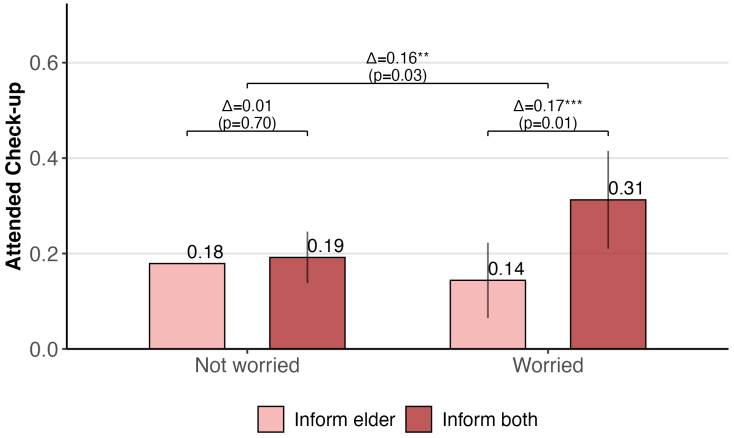
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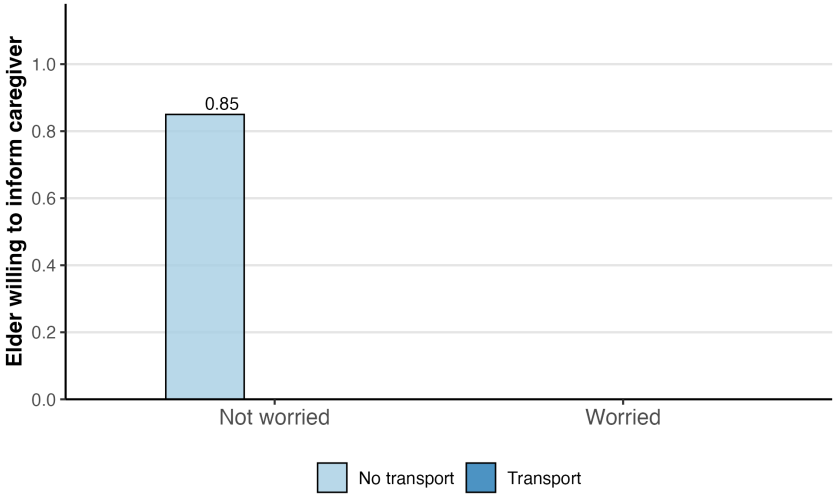


Ruling out alternative explanations

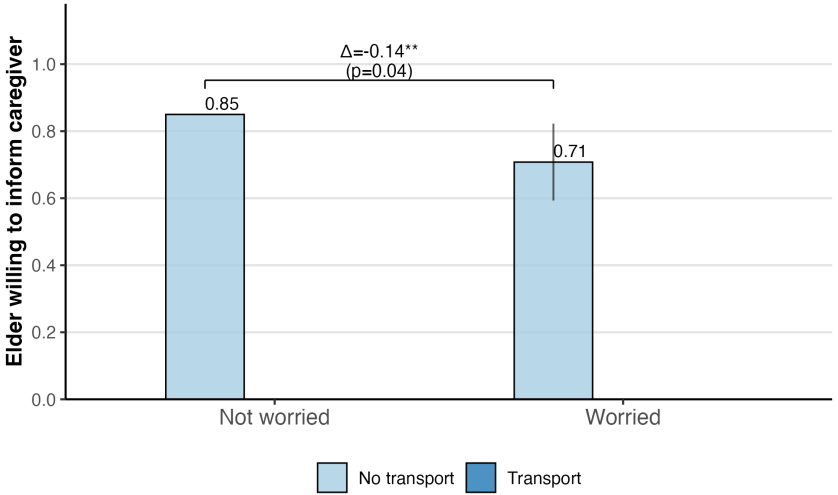
Our results are not explained by...

- Differences in elder or caregiver characteristics Elder traits by worry
- Higher actual costs of attending With interacted controls
- Higher asking costs (rejection sensitivity, logistical) Direct vs Indirect Info Sharing
- Less frequent interaction with caregivers Closeness by worry

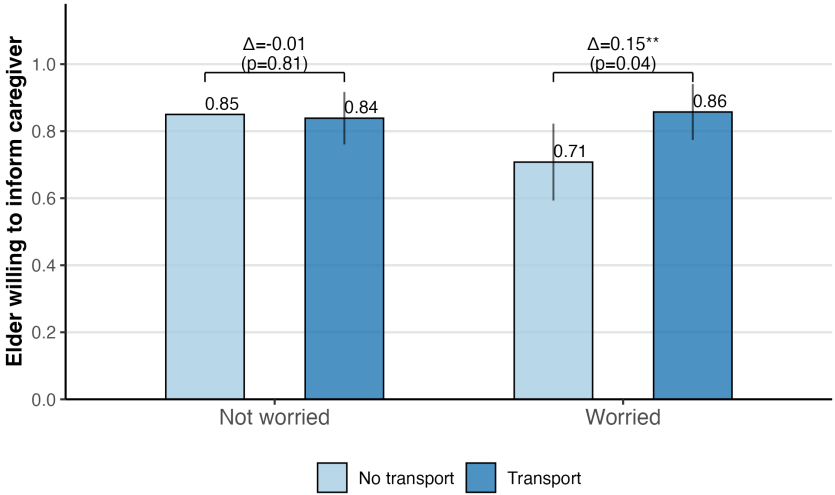
We directly measure elder willingness to inform caregiver about check-up



Worried elders are 14pp less willing to inform caregivers about check-ups



But worried elders respond to a decrease in caregiver costs (+15pp)



Upshot: Household experiment shows that caregivers help when informed

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- $\Rightarrow$ Elders overestimate pressure imposed by informing the caregiver

Taking stock: Worry about burden captures an important relationship dynamic

- Households are aging, making **caregiving burden a key issue**
  - Elders worry about burden, which decreases health take-up
  - ... despite caregivers being willing to help
- Mutual “caring" can **amplify information frictions and lower health take-up**
  - Concern for others can backfire when others’ preferences unknown
- **Policy can help**: Sharing info directly with caregivers increases take-up
  - Treatment effects are large, comparable to providing free transport

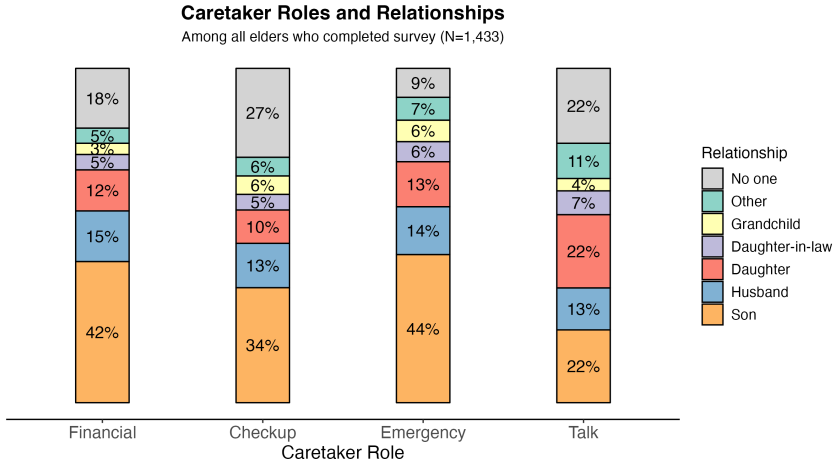
Thank you!

wangjm@mit.edu

Our sample consists of elderly women aged 60-75 in rural Tamil Nadu

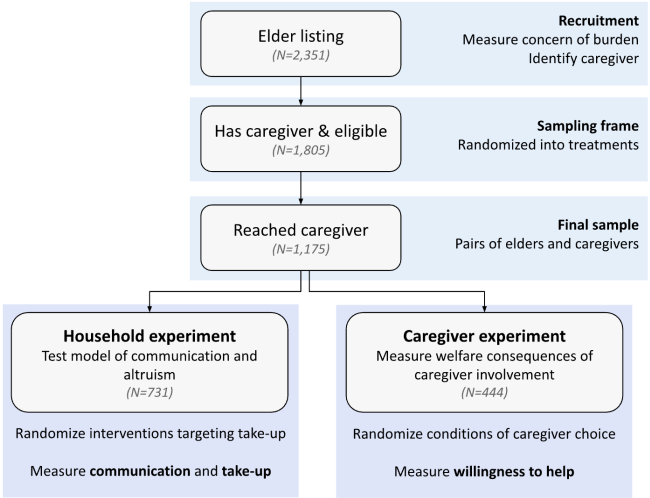
- Median age 68 years old, about half are widowed (52%)
- **Most are mobility impaired** (77% difficulty walking >1km, 58% leaving home)
- 82% of elders live with their caregiver
- Most have had some kind of check-up in the last year (83%)
- 16% have gotten heart check-up in the last year
- 93% were **interested in attending** the check-up

Elders primarily depend on sons and husbands for health-related support

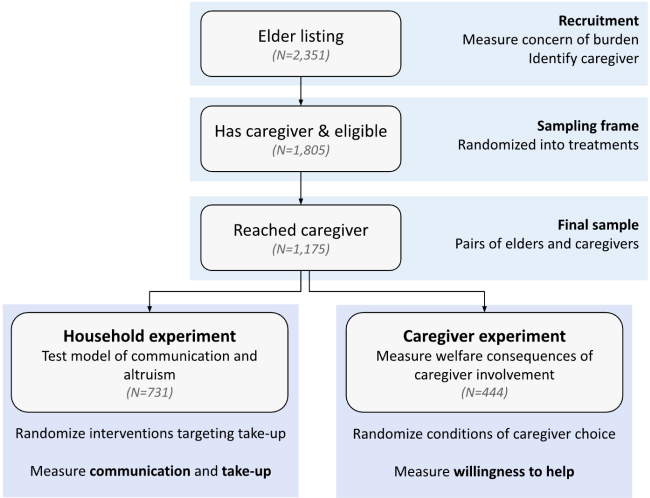


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Sampling: We recruit pairs of elders + caregivers for a household survey



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Elders worried about burden are more mobility constrained and dependent

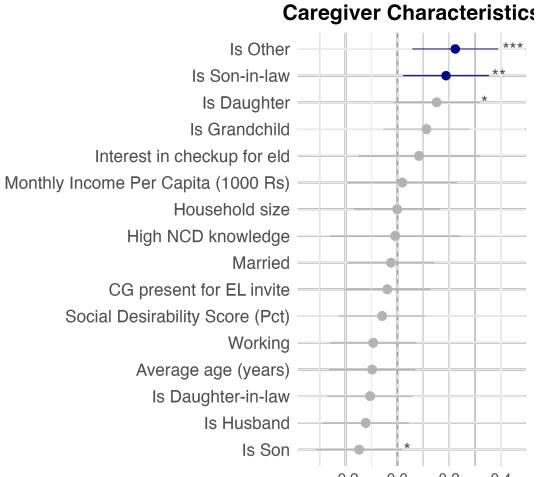
Their caregivers are more likely to be daughters, but otherwise similar in characteristics

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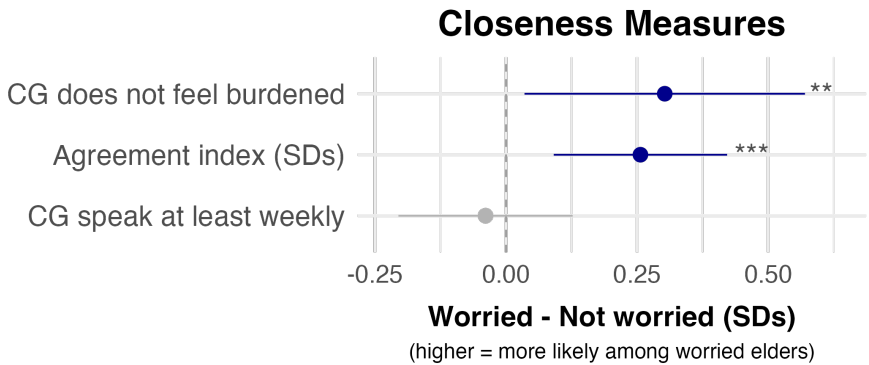
Elders worried about burden have caregivers who are less likely to be sons

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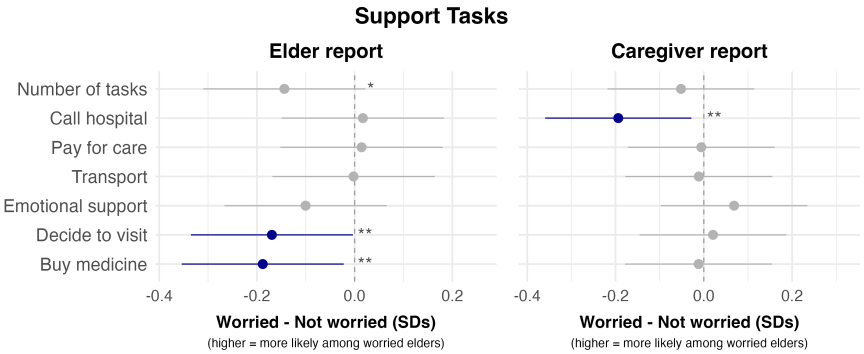
Elders worried about burden have caregivers who are less likely to feel burdened

Back



Worried elders do not receive more support from caregivers

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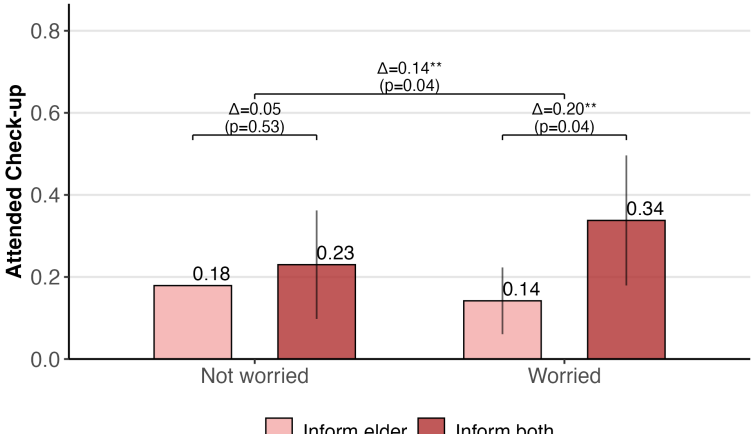
Worried elders do not differ in time to mention health issues (pain, sickness)

Back



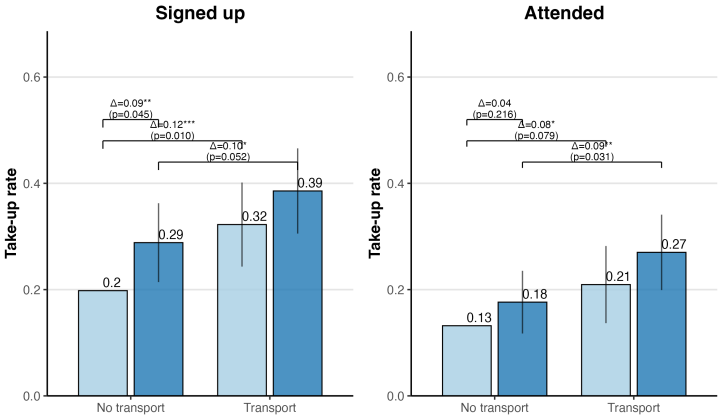
Heterogeneity persists even when controlling for imbalanced characteristics, interacted with treatment

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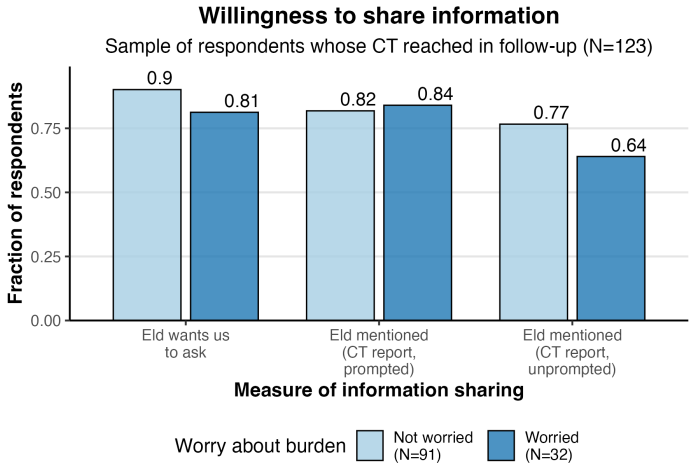
Interaction effect of transport and tell both is positive but not statistically significant

Not powered to test for interaction effects



Direct and indirect willingness to inform rates are similar

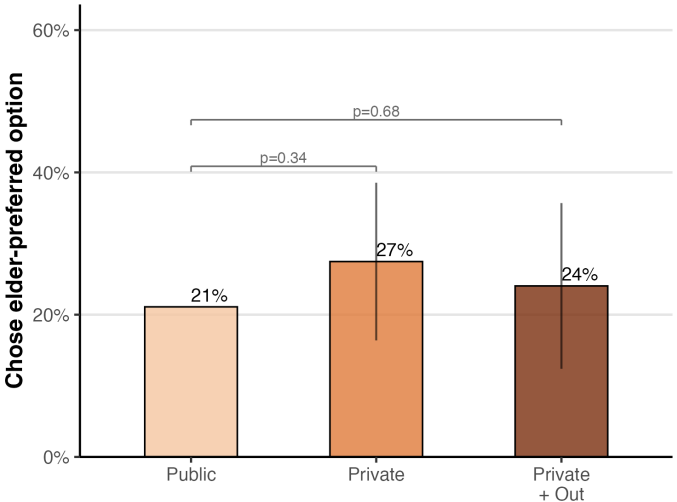
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Testing for pressure: We randomize observability of caregiver decision to help

- Offer caregivers a check-up for the elder
- Outcome: Among caregivers who agree, offer two types of **transport support**
  - Choose option preferred by elders (reimbursement)
  - Choose option preferred by caregivers (arranged)
- Randomize condition under which caregiver chooses
  - **Public.** Baseline condition
  - **Private.** Removes social pressure (surveyor, elder)
  - **Private + Out.** Removes social and psychological pressure (e.g. guilt, self-image)[1]

Caregivers help at similar rates, even when offered an out

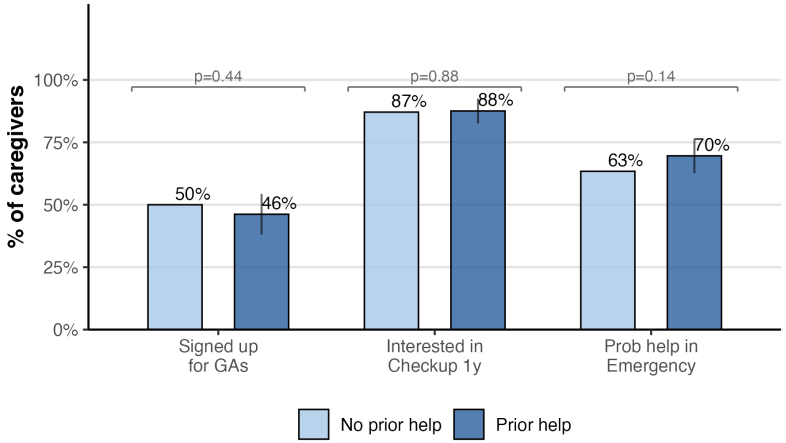


Testing for crowd-out: Randomize whether caregiver has already provided help

- Outcomes: Willingness to help on other tasks
  1. Sign up for group activities (20 Rs)
  2. Want to learn more about general checkup in 1y
  3. Willingness to help in emergency (5-point scale)
- Randomize whether check-up is offered **before** vs **after** measuring outcomes
  - 63% of caregivers sign up for the check-up[2]

Crowd-out model

Caregivers do not reduce help in other domains



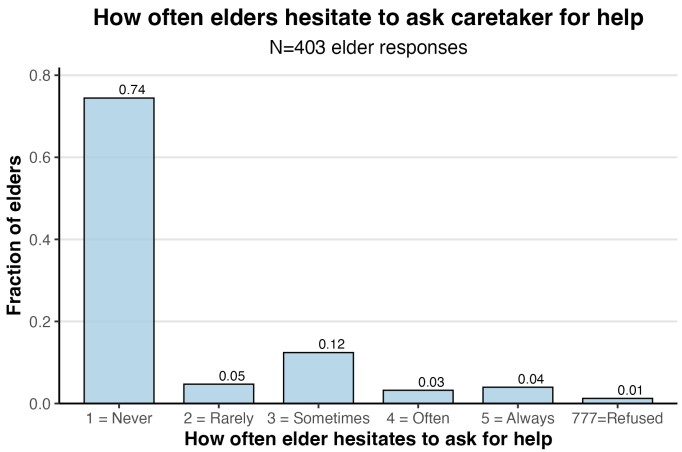
Treatment = invited to checkup before GAs. 90% confidence intervals.

No find evidence of differential crowd-out for caregivers of worried elders

	Dependent variable:		
	GA Signup	Interested in Checkup 1y	Emerg: Prob help
	(1)	(2)	(3)
Prior Help Committed	−0.037	0.016	0.073
	(0.061)	(0.037)	(0.052)
	p = 0.538	p = 0.674	p = 0.155
Worry	0.045	0.076	0.001
	(0.078)	(0.048)	(0.067)
	p = 0.567	p = 0.113	p = 0.986
Prior Help * Worry	0.012	−0.035	−0.024
	(0.105)	(0.064)	(0.089)
	p = 0.910	p = 0.584	p = 0.792
Control Mean	0.5	0.87	0.63
Strata FEs	Y	Y	Y
Controls (LASSO)	Y	Y	Y
Observations	444	444	444
R^2	0.275	0.326	0.414
Note:			$^{*}p<0.1$; $^{**}p<0.05$; $^{***}p<0.01$

Back

Most elders report not hesitating to ask when they need help with something (74%)



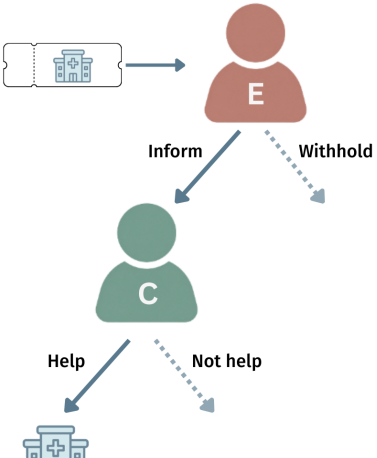
Among those who ever hesitate to ask, burden is most common reason (70%)

Few mention rationing their asks to the caregiver (8%)

Table: Elder reasons for not asking for help

variable	N	Fraction
I do not want to burden or trouble them	68	0.69
They may be upset with me for asking	36	0.37
I think they will say no	22	0.22
I don’t like to feel dependent	13	0.13
I may need help for something urgent later	8	0.08
Don’t know	1	0.01
Other, specify	1	0.01

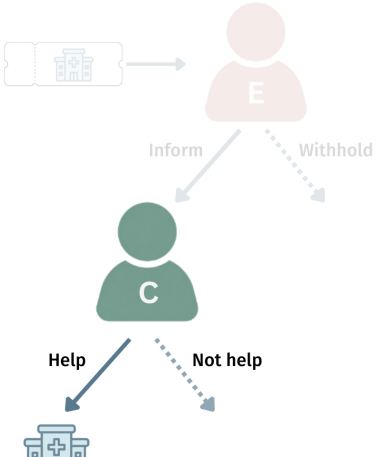
Model sketch



Elder wants to visit clinic, but needs caregiver

1. Elder decides whether to inform
   - Key friction: Elder doesn't know caregiver motives, may worry (care + pressure)
2. Caregiver decides whether to help
   - Caregiver helps out of caring or pressure

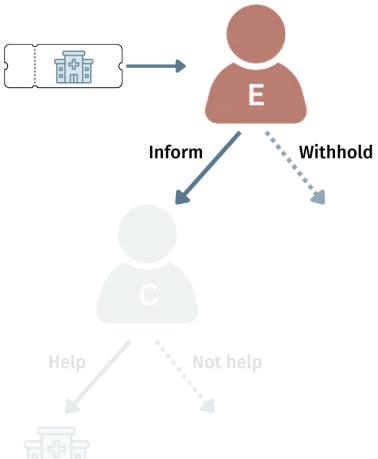
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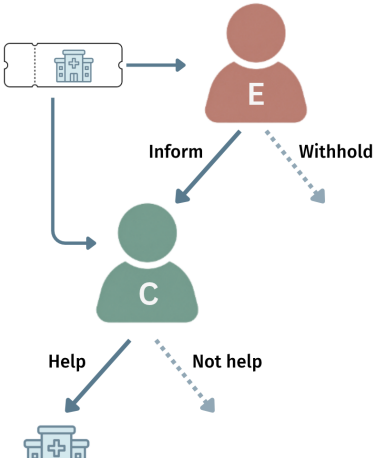
Model sketch



Elder wants to visit clinic, but needs caregiver

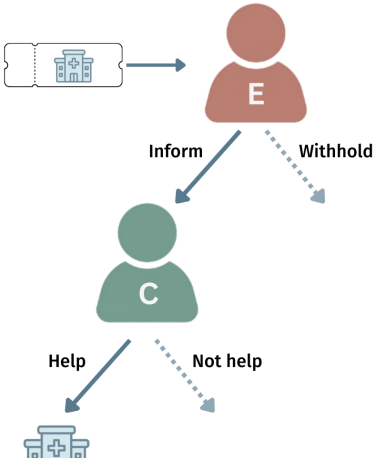
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Testable predictions from the model



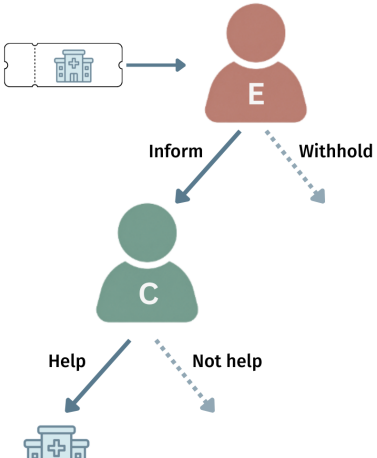
1. Informing caregiver can affect take-up

Testable predictions from the model



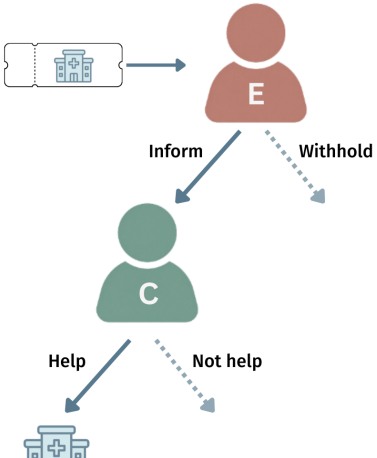
1. Informing caregiver can affect take-up
2. Communication varies with elder worry

Testable predictions from the model



1. Informing caregiver can affect take-up
2. Communication varies with elder worry
    a. Caring: Worried elders respond to caregiver costs
    b. Pressure (perceived): Worried elders inform less when they believe there is pressure

Testable predictions from the model



1. Informing caregiver can affect take-up
2. Communication varies with elder worry
    a. Caring: Worried elders respond to caregiver costs
    b. Pressure (perceived): Worried elders inform less when they believe there is pressure
3. Caregiver help responds to observability when caregiver is pressured